

## M01 MENTAL HEALTH PROMOTION | P

**Intent :** Promote mental health and well-being through the provision of supportive programs, policies and resources.

**Summary :** This WELL feature requires projects to provide programs, policies and resources that support and promote occupant mental health.

**Issue :** Recent estimates indicate that common mental health conditions (e.g., anxiety, depression, substance use) are experienced by 18% of adults over a 12-month period, and 30% of adults over their lifetime.<sup>1</sup> Depression is the leading cause of disability in the world, and depression and anxiety alone cost the global economy an estimated \$1 trillion due to lost productivity, yet global annual spending on mental health is less than \$2 per person.<sup>2,3</sup> Failure to address mental health in the workplace can contribute to high turnover, presenteeism and absenteeism, repetitive recruitment and training costs, increased use of drug plans, disability claims, sick leave, employee assistance plans (EAPs) and other costly supports, increased accidents and injuries, and burnout.<sup>4-6</sup>

**Solutions :** Given the high prevalence of mental health conditions among the working population, the workplace is an important target for promotion, prevention and intervention efforts.<sup>7,8</sup> Strategies include increased organizational support, enhancement of mental health literacy and reduction of stigma, which have been shown to help people recognize signs of poor mental health and encourage help-seeking behavior.<sup>8,9</sup> Improving opportunities for restoration through mindfulness programming and optimal sleep can have a marked impact on mental well-being, including relief from negative symptoms associated with anxiety, depression, pain and stress, as well as enhancements in overall perceived health.<sup>10-13</sup>

### PART 1 PROMOTE MENTAL HEALTH AND WELL-BEING

*For All Spaces:*

The following requirements are met:

- a. At least two of the following are available to all employees and students at no additional cost:
  1. Education or awareness efforts on mental health and well-being, offered quarterly, either in-person or virtually (e.g., webcast on stress management, presentation on mindfulness, email on healthy sleep habits).<sup>14,15</sup>
  2. Trainings or courses related to mental health and well-being, offered annually, either in-person or virtually (e.g., Mental Health First Aid, stress management training).<sup>14,15</sup>
  3. Mindfulness or restorative programming, offered weekly, either in-person or virtually (e.g., ongoing access to guided meditation application, weekly yoga classes).<sup>7</sup>
  4. Policy that establishes healthy working hours, outlining the maximum hours to be worked per 24-hour and seven-day period.<sup>16</sup>
  5. Dedicated space for restoration and relaxation, with an accompanying policy permitting breaks during work or school hours.<sup>16,17</sup>
- b. Annual communication (e.g., email, online module, in-person training) is provided to all employees and students, and onboarding communications are provided to all new employees and students, specifically addressing all mental health and well-being benefits, resources and programs available through the project or organization.

**Note :** Projects may achieve points in optimizations that overlap with strategies listed in Part 1.a.

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## M02 NATURE AND PLACE | P

**Intent :** Support occupant well-being by incorporating the natural environment throughout the project and integrating design strategies that celebrate the project's unique identity.

**Summary :** This WELL feature requires the integration of nature throughout the project, as well as design strategies that celebrate the project's unique identity and inspires human delight.

**Issue :** Humans are increasingly living in environments where they have insufficient exposure to nature.<sup>1,2</sup> Natural elements, such as plants and daylight, have been linked with health-promoting benefits, including decreased levels of depression and anxiety, increased attentional capacity, better recovery from job stress and illness, increased pain tolerance and increased psychological well-being.<sup>3-5</sup> The incorporation of plants in the work environment is linked with improved employee morale, job satisfaction, and objective and subjective measures of productivity and with decreased absenteeism.<sup>4,6-8</sup> Finally, the presence of water, natural light and nature views can impact mood, memory and performance in the workplace.<sup>9-11</sup> A dose-response relationship has been found with exposure to indoor nature, with studies showing that as workday nature contact increased, perceived job stress, subjective health complaints and sickness absence decreased.<sup>5,12,13</sup>

**Solutions :** Incorporating natural elements into buildings can support occupant relief from stress and mental fatigue, as well as help establish a sense of place.<sup>14</sup> The benefits of nature access can be achieved through numerous pathways such as direct (e.g., plants in the office), indirect (e.g., window views) or representational (e.g., photographs).<sup>4</sup> Additionally, incorporating other key aesthetic elements, such as local culture, materials and art can help celebrate the project's unique identity and further enrich the space for occupants and visitors.

### PART 1 PROVIDE CONNECTION TO NATURE

*For All Spaces except Dwelling Units:*

The project integrates the following throughout the space, including common circulation routes, shared seating areas and rooms (e.g., conference rooms, common spaces) and workstations (as applicable):

- a. Natural materials, patterns, shapes, images or sounds.<sup>9,15</sup>
- b. At least one of the following:
  1. Plants (e.g., potted plants, plant walls).<sup>9,15</sup>
  2. Water (e.g., fountain, pond, fish tank).<sup>9,15</sup>
  3. Nature views.<sup>9,15</sup>

### PART 2 PROVIDE CONNECTION TO PLACE

*For All Spaces except Dwelling Units:*

The project integrates design elements that address the following:

- a. Celebration of culture (e.g., culture of occupants, workplace, surrounding community).<sup>16</sup>
- b. Celebration of place (e.g., local architecture, materials, flora, artists).<sup>16</sup>
- c. Integration of art.<sup>16</sup>
- d. Human delight.<sup>16</sup>

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### M03 MENTAL HEALTH SERVICES | O (MAX : 4 PT )

**Intent :** To increase awareness of mental health conditions, as well as offer supportive workplace services and accommodations for those living with such conditions.

**Summary :** This WELL feature requires projects to support occupant mental health through the provision of programs and resources.

**Issue :** Obtaining appropriate treatment for mental health conditions remains a global barrier. It is estimated that 76-85% of people in low- and middle-income countries and 30-50% in high-income countries receive no treatment, and those who do receive help often face issues with poor quality care.<sup>12</sup> Compared to other illnesses, mental health conditions have longer delays in treatment onset.<sup>3,4</sup> Many factors contribute to this gap, including a lack of equity in healthcare coverage between benefits for mental health, substance abuse and addiction, and benefits for other medical needs, as well as insufficient mental health promotion to increase awareness and minimize the stigma associated with mental health conditions.<sup>5,6</sup> This lack of treatment has major public health and economic consequences: individuals with depression miss an average of 4.8 workdays and experience 11.5 days of reduced productivity in a three-month period, and people living with mental health conditions are overall less likely to receive high quality medical care and preventive health services (e.g., immunizations, cancer screening, tobacco cessation support).<sup>7,8</sup> Furthermore, many individuals who go through emergency situations (e.g., natural disaster) experience psychological distress, resulting in depression, anxiety, feelings of hopelessness, fatigue, irritability or anger.<sup>9,10</sup> These impacts can be exacerbated by added stressors during emergencies such as social isolation, economic hardship or grief, reinforcing the need for adequate access to mental health services.<sup>9,10</sup>

**Solutions :** Equitable access to screening and mental health services can help encourage care utilization, support early diagnosis and overall mitigate poor mental health outcomes.<sup>5</sup> Enhanced social support and adjustments to the work environment can also help enable a successful return for employees coming back from leave due to a mental health condition.<sup>11,12</sup> Additionally, providing access to mental health services during and after emergency situations, such as psychological first aid, crisis counseling and bereavement counseling, is critical to supporting employee short-term recovery and long-term productivity, functioning and well-being.<sup>9,10,13,14</sup>

#### PART 1 OFFER MENTAL HEALTH SCREENING (MAX : 1 PT )

*For All Spaces:*

The project or organization makes a clinical assessment screening tool (e.g., self-assessment, screening tool administered by a professional) for common mental health conditions available to all employees and students either in-person or virtually, at no additional cost. The tool meets the following requirements:

- a. Addresses, at minimum, stress, depression, anxiety and substance use.
- b. Provides confidentiality by leveraging a licensed mental health professional, third party organization, online screening or health insurance offering.
- c. Includes directed feedback and/or guidance on interpretation of results and provides next steps for those who screen positive or at-risk.<sup>6,15</sup>

#### PART 2 OFFER MENTAL HEALTH SERVICES (MAX : 1 PT )

*For All Spaces:*

The project or organization makes a mental health benefits policy available to all eligible employees that meets following requirements:

- a.
  - Mental health support is available at no additional cost or subsidized and covers the following at a minimum:
    1. Clinical screening and referral to licensed mental health professionals and support resources.<sup>6</sup>
    2. Inpatient treatment (e.g., residential programs, hospitalization).<sup>6</sup>
    3. Outpatient treatment, including options for telemental health services (e.g., in-person therapy, online therapy).<sup>6,16</sup>
    4. Prescription medication.<sup>6</sup>
- b. Mental health parity in health service coverage.<sup>6</sup>
- c. Information on benefits coverage and how to access mental health services and community resources is easily and confidentially available (e.g., via a health portal or employee website).<sup>6</sup>
- d. Confidential benefits consultation with people who are clearly identified and qualified (e.g., benefits counselor, human resources representative) is made available.

#### PART 3 OFFER EMPLOYEE MENTAL HEALTH SUPPORT (MAX : 1 PT )

*For All Spaces:*

The project or organization has a mental health policy and the benefits within are made available to all employees without a need to disclose the underlying health reason. The mental health policy meets the following requirements:

- a. Sick leave may be used for mental health needs (e.g., appointments).<sup>12,17,18</sup>
- b. Short- or long-term leave may be used for mental health needs, with the option of a phased integration back to work after returning from leave.<sup>18,19</sup>
- c. Increased interpersonal support (e.g., manager support with prioritizing and managing workloads, increased frequency of one-on-one check-ins).<sup>12</sup>
- d. Adjustment of work schedule to support mental health needs (e.g., appointments, start/end times).<sup>12,17,18</sup>
- e. Adjustment of the workplace to support mental health (e.g., moving a workstation to a busier or a quieter area, providing a quiet space for breaks, providing earplugs or headphones, increasing personal space, providing the

ability to work from home).<sup>12,17,18</sup>

#### PART 4 B SUPPORT MENTAL HEALTH RECOVERY (MAX : 1 PT )

*For All Spaces:*

Projects offer mental health services and resources to support recovery from a traumatic event to all employees at no additional cost or subsidized, on-site, in-person within 400 m of the project boundary or virtually, including at least three of the following:

- a. Crisis counseling or trauma-focused psychotherapy with qualified mental health professionals.
- b. Psychological first aid (PFA) training offered to all employees and/or required for manager-level employees.<sup>20</sup>
- c. Bereavement counseling and materials on coping with grief, including resources for returning to work after a loss.
- d. Information on benefits coverage and how to access additional mental health services, made conveniently and confidentially accessible to employees.

**Note :** This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.

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## M04 MENTAL HEALTH EDUCATION | O (MAX : 2 PT )

**Intent :** Promote mental health awareness and education through the provision of mental health-focused training or education offerings.

**Summary :**

This WELL feature requires projects to provide education and training to employees and managers to help them better understand how to manage their own mental health and support others.

**Issue :** There is a need for increased knowledge regarding causes and recognition of different types of mental health conditions, beliefs about treatment for mental disorders and reduction in stigma against those with mental health conditions.<sup>1,2</sup> The combination of these factors leads to delays in recognition and help-seeking, hinders public acceptance of evidence-based mental health care and denies those with mental health conditions appropriate support from their communities.<sup>2</sup>

**Solutions :** Workplace mental health education interventions can help create a more supportive work environment. Enhancing knowledge and awareness can reduce stigmatizing attitudes and discrimination, as well as improve employee mental health via increased and potentially earlier help-seeking.<sup>3</sup> Education and anti-stigma interventions have been shown to have a positive impact on participants' own general mental health,<sup>3</sup> as well as their behavior toward those with mental health conditions, such as: enhancing perceived confidence and self-efficacy in identifying and supporting a person with a mental health condition, increased likelihood of advising those in need to seek professional help and greater readiness to provide help in a mental health situation.<sup>3</sup> A systematic review of Mental Health First Aid training demonstrated increased participants' knowledge regarding mental health, decreased negative attitudes, increased supportive behavior and confidence in helping others and has been shown to be highly acceptable in a workplace setting.<sup>4,5</sup>

### PART 1 OFFER MENTAL HEALTH EDUCATION (MAX : 1 PT )

*For All Spaces:*

Trainings (in the form of education seminars, workshops or classes) are offered at least twice per year or on demand to regular occupants and meet the following requirements:

- a. Address at least two of the following topics:
  1. Managing personal mental health and well-being, covering topics such as developing mentally healthy habits and self-care practices, fostering relationships and social connections and managing mental health at work.<sup>6</sup>
  2. Common mental health conditions or concerns, covering, at minimum, depression, anxiety, substance use, stress, and burnout, and loneliness and social isolation.
  3. Signs and symptoms of mental health distress, including how to identify emotional distress and appropriately respond (e.g., Mental Health First Aid).<sup>6</sup>
- b. Provided in-person or virtually (live or recorded), in group or individual settings, and through vendors, on-site staff, health insurance plans, community groups or other qualified programs (e.g., Mental Health First Aid).<sup>7</sup>

### PART 2 OFFER MENTAL HEALTH EDUCATION FOR MANAGERS (MAX : 1 PT )

*For All Spaces:*

All managers undergo annual mental health training (in the form of education seminars, workshops or classes) that meets the following requirements:

- a. Addresses at least three of the following topics:
  1. Identifying and reducing workplace stress-related issues (e.g., conducting performance reviews, effective communication skills, personnel management, conflict resolution).<sup>7</sup>
  2. Recognizing common mental health conditions or concerns, covering, at a minimum, depression, anxiety, substance use, stress and burnout, and loneliness and social isolation.<sup>7</sup>
  3. Supporting employee mental well-being using strategies to prevent burnout, low motivation, fatigue, poor work-life balance and other work-related stress issues.<sup>8</sup>
  4. Recognizing employee mental health concerns or crises, including increasing awareness of workplace and community resources available to employees.<sup>7</sup>
- b. Provided in-person or virtually (live or recorded), in group or individual settings, and through vendors, on-site staff, health insurance plans, community groups or other qualified programs (e.g., Mental Health First Aid).<sup>7</sup>

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## M05 STRESS MANAGEMENT | O (MAX : 2 PT )

**Intent :** Identify areas of stress within the workplace and create a plan for stress management.

**Summary :** This WELL feature requires projects to assess stress within the organization and create a plan for relieving or modifying sources of stress.

**Issue :** Stress is directly linked to seven of the ten leading causes of death in the world, as well as numerous negative health consequences, including obesity, high cholesterol, muscle tension and backache, migraines and chronic headaches and poor recovery from illness.<sup>1-4</sup> Stress is also a predictor of adverse mental health outcomes, such as depression, anxiety, substance use, suicide, emotional exhaustion and burnout.<sup>3-5</sup> When stressed, individuals are less likely to engage in other key health behaviors, including smoking cessation, healthful eating and physical activity.<sup>3</sup> Employee stress is incredibly common, with recent studies reporting 94% of workers feel stress on the job and 25% report work as their number one stressor.<sup>6,3</sup> Stressed employees are more likely to quit, be involved in an accident, experience reduced performance and incur an average of nearly 46% higher health care expenditures compared to less-stressed peers.<sup>7</sup> Employees who experience stress are more likely to miss work, resulting in an estimated one million worker absences per day due to stress.<sup>3</sup>

**Solutions :** Numerous factors increase the likelihood of workplace stress, such as low support from supervisors and colleagues, little control over work processes, unmanageable and high demands, concern over a lack of job security and low opportunity for advancement or professional development.<sup>3</sup> Stress and its associated risks can be reduced through interventions that adjust job stressors, such as changes in operations, increases in co-worker and supervisor support and training employees in developing resilience to withstand job stressors.<sup>4,5</sup>

### PART 1 DEVELOP STRESS MANAGEMENT PLAN (MAX : 2 PT )

*For All Spaces:*

The project or organization develops a stress management plan by completing the following:

- a. Assess at least three of the organization- or project-wide metrics below:
  1. Frequency of employees working more than 48 hours per seven-day period.<sup>8</sup>
  2. Frequency of absenteeism amongst employees (i.e., use of paid or unpaid time off due to disability or illness).
  3. Frequency of employees not using allocated paid time off.
  4. Frequency of performance issues amongst employees.
  5. Employee retention and turnover rates.
  6. Responses to employee satisfaction surveys that indicate high levels of stress or burnout
- b. Identify opportunities for improvement, covering the topics below:
  1. Organizational change to address employee stress (e.g., adjustments to work environment, shifts in work processes, workload, management or staffing).<sup>9</sup>
  2. Employee participation in organizational decisions regarding work-related issues that may affect stress (e.g., work environment, processes, scheduling).<sup>9</sup>
- c. Create and implement a stress management plan that identifies the following:
  1. The person leading implementation of the plan.<sup>10</sup>
  2. The changes that are to be completed as part of the plan.<sup>10</sup>
  3. Who will be impacted by those changes.<sup>10</sup>
  4. When and how the changes will be implemented.<sup>10</sup>
  5. Confirmation of support from executive leadership.<sup>4</sup>

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## M06 RESTORATIVE OPPORTUNITIES | O (MAX : 2 PT )

**Intent :** Support employee well-being by providing opportunities for recovery and restoration within and outside the workplace.

**Summary :**

This WELL feature requires projects to support recovery and restoration from work and to encourage a healthy work-life balance by providing opportunities for sleep, breaks throughout the workday and sufficient paid time off.

**Issue :** Long working hours and insufficient opportunities for recovery are associated with numerous adverse outcomes, including cardiovascular and immunologic reactions, reduced sleep quality and duration and increased risk for stress, burnout, excessive alcohol use and poor diet.<sup>1,2</sup> Insufficient sleep, in particular, is associated with numerous negative outcomes, such as increased pain and anxiety, impaired performance and productivity and increased errors and risk of accidents.<sup>3-5</sup> Long hours are also connected to reductions in employee creativity and morale.<sup>2,6</sup> Research shows that working over weekends inhibits recovery from work-related stress and that ongoing periods without vacation have adverse health effects, including increased cardiovascular risk.<sup>5</sup>

**Solutions :** Employees need sufficient opportunities to psychologically detach and recover during non-work hours, such as weekends, vacations and evenings. <sup>5</sup> Findings indicate that mentally distancing oneself from work and engaging in restorative activities on a day-to-day basis is linked to employee well-being, including higher life satisfaction and mood, maintained workplace performance, lower burnout and fewer health complaints.<sup>5</sup> Targeted interventions, such as limits on working hours and schedule, can help support individuals in achieving sufficient and high-quality sleep.<sup>7,8</sup> Research indicates that regular vacations may have a protective effect against chronic work stress by providing a sustained period of relief from daily stressors, demands and routines.<sup>9,10</sup> Short afternoon naps also provide an opportunity for restoration during the workday, leading to benefits in mood, alertness and cognitive performance, such as productivity and learning.<sup>11,12</sup>

### PART 1 SUPPORT HEALTHY WORKING HOURS (MAX : 1 PT )

*For All Spaces:*

The project or organization has a policy on schedules and time off that meets the following requirements:

- a. For all employees:
  1. A minimum of 11 consecutive hours off from work is available per 24-hour period.<sup>13,14</sup>
  2. A minimum of 24 consecutive hours off from work is available per 7-day period.<sup>14</sup>
  3. Employees who engage in shift work are provided a minimum 48-hour advance notice of shift changes.
- b. For all eligible employees:
  1. A minimum of 20 days of paid time off per calendar year (not including paid sick days or public holidays).<sup>14</sup>
  2. Work and work-related communications are not required during paid time off.<sup>5</sup>
  3. Sick, vacation, floating holiday, personal and all other employer-provided days off from work are clearly defined.
  4. Accrual policy is defined, including whether rollover days are allowed and date by which rollover days must be used.
- c. For students in secondary schools (if applicable), the school day starts no earlier than 8:30 a.m.<sup>17</sup>

### PART 2 PROVIDE NAP POLICY AND SPACE (MAX : 1 PT )

*For All Spaces:*

*1: Nap policy*

The following requirement is met for all eligible employees:

- a. Employees are allowed to engage in at least one nap or rest break of at least 30 minutes during the workday (not to be combined with designated time for meal breaks).<sup>13,18</sup>

*2: Nap space*

The following requirements are met for all eligible employees:

- a.
  - Access to at least one acoustically and visually separated environment located in a designated quiet zone.
- b. At least one fully reclining furniture option (e.g., bed, daybed, couch, chair, cushioned roll-out mat, nap pod) for every 100 eligible employees.<sup>13,18</sup>

**Note :**

Projects should refer to feature S01 Sound Mapping, Part 1: Label Acoustical Zones for all designated quiet zones.

Projects can utilize one room as a combined nap space (M06) and restorative space (M07). Projects will only receive points for both features, if all requirements for M06 Part 2 and M07 Part 1 have been met.

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## M07 RESTORATIVE SPACES | O (MAX : 1 PT)

**Intent :** Support access to spaces that promote restoration and relief from mental fatigue or stress.

**Summary :** This WELL feature requires projects to provide spaces that promote a restorative environment and encourage relief from mental fatigue and stress.

**Issue :** Work-induced fatigue is common among office workers, with research indicating that over 20% of the workforce experiences prolonged fatigue.<sup>1,2</sup> During the workday, individuals experience fatigue caused by a depletion of physical and mental resources, resulting in an accumulated need to recover from mental fatigue and stress.<sup>1</sup> The experience of prolonged fatigue combined with a high need for mental recovery is strongly associated with psychological distress, including decreased mental acuity, deficits in motivation and irritability caused by prolonged exposure to stress.<sup>1,2</sup>

**Solutions :** By providing restorative spaces for individuals to step away from the stress of the office environment, recharge and refocus, employers can help alleviate the negative effects associated with workplace fatigue or mental depletion.<sup>3</sup> Through incorporation of nature, among other restorative elements, these spaces can help relieve stress and mental fatigue, support focus and encourage overall mental well-being.<sup>4</sup> Exposure to plants and other natural elements has been linked with decreased levels of diastolic blood pressure, depression and anxiety, increased attentional capacity, better recovery from job stress and increased psychological well-being.<sup>5,6</sup> Nature interaction has also been shown to support recovery from illness and increase pain tolerance.<sup>5,6</sup> Outdoor spaces can also be used to promote calm and encourage restorative activities. Both indoor and outdoor spaces can also be used for individuals with a wide variety of beliefs, religions and traditions, including prayer and meditation.

### PART 1 PROVIDE RESTORATIVE SPACE (MAX : 1 PT)

*For All Spaces:*

*1: Restorative space*

The project provides at least one indoor or outdoor space for all regular occupants. The space may be made up of a single space or multiple spaces that meet the following requirements:

- a. The main purpose is for relaxation and restoration. Space may serve multiple functions but is not to be used for work.
- b. Minimum size of 7 m<sup>2</sup> plus 0.1 m<sup>2</sup> per regular occupant or 186 m<sup>2</sup> whichever is smaller.
- c. Provides a calming and comfortable environment by incorporating at least five of the following:
  1. Adjustable lighting (e.g., dimmable light levels for indoor spaces).<sup>7</sup>
  2. Sound interventions (e.g., water feature, natural sounds, sound masking).<sup>8,9</sup>
  3. Thermal control (e.g., fans, shading).<sup>10</sup>
  4. Seating arrangements that accommodate a range of user preferences and activities (e.g., movable lightweight chairs, cushions, mats).<sup>7</sup>
  5. Nature or natural elements.<sup>11-13</sup>
  6. Subdued colors, textures and forms.<sup>14-15</sup>
  7. Visual privacy.<sup>16</sup>
- d. Includes signage, education materials or other resources explaining its purpose and intended use.

**Note :** If restorative space is provided only outdoors, it must be functional year-round.

*2: Workday breaks*

The project encourages the use of restorative space(s) through the following:

- a. Paid breaks away from the workstation for all employees.<sup>17</sup>

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## M08 RESTORATIVE PROGRAMMING | O (MAX : 1 PT )

**Intent :** Support access to programs that promote restoration and relief from mental fatigue or stress.

**Summary :** This WELL feature requires projects to develop ongoing programming for occupants that is focused on relaxation, restoration or mindfulness, such as meditation or mindful movement.

**Issue :** Workplace stress is incredibly common. In the European Union, for example, an estimated one in three workers experience work-related stress.<sup>1</sup> Exposure to stressful work conditions is associated with poor mental and physical health.<sup>2</sup> Highly stressed employees are at risk for numerous negative outcomes, including decreased productivity, greater absenteeism, increased occupational injury and higher overall medical expenditures.<sup>3-5</sup>

**Solutions :** Introducing mindfulness into the workplace has been shown to lower employee stress, as well as improve focus, clarity of thinking, decision-making and emotional intelligence.<sup>6,7</sup> Meditation interventions targeting workers have been found to be effective at reducing work-associated stress, depression, anxiety and burnout, as well as improving mood and sleep quality.<sup>8-11</sup> A recent systematic review concluded that mindfulness meditation can help reduce negative dimensions of psychological stress, including improving anxiety, depression, pain, stress and overall mental health.<sup>12</sup> Mindfulness-based stress reduction, a widely disseminated and frequently used practice, has been shown to reduce symptoms of stress, depression and anxiety, and enhance self-esteem, body image, mood and coping with other health problems, such as chronic pain, fatigue, stress and insomnia.<sup>13-15</sup> Mindfulness-based practices, whether formal or informal, can help improve employee focus and productivity, support stress management and reduce employer costs through improvements to overall health and well-being.<sup>7-11</sup>

### PART 1 PROVIDE RESTORATIVE PROGRAMMING (MAX : 1 PT )

*For All Spaces:*

At least two of the following are offered to all eligible employees at no cost or subsidized by at least 50%:

- a. Mindfulness training course (e.g., eight-week mindfulness-based stress reduction course (MBSR)) offered live, either in-person or virtually, by a qualified mindfulness instructor at least twice a year, that meets the following:
  1. Defines mindfulness and its component parts.
  2. Covers relevant research on mindfulness.
  3. Teaches both formal practices (e.g., mindfulness meditation, yoga postures) and informal practices (e.g., mindful eating, mindful listening) that can be applied during the workday.
- b. Mindfulness programming (e.g., guided meditation, yoga) offered live, either in-person or virtually, at least once a week in a designated quiet zone.
- c. Digital mindfulness offerings (e.g., guided meditation application). Employees have unlimited access to at least one digital offering and access to at least one designated quiet zone.

**Note :** Refer to feature S01 Sound Mapping, Part 1: Label Acoustical Zones for all designated quiet zones.

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## M09 ENHANCED ACCESS TO NATURE | O (MAX : 2 PT )

**Intent :** Support access to nature beyond M02: Access to Nature and Beauty, by further incorporating nature through interior design, exterior design and access to nearby nature.

**Summary :** This WELL feature requires the integration of nature and natural elements into the interior and exterior of the project, as well as the provision of nature views and nearby nature, such as green and blue spaces.

**Issue :** Exposure to plants and other natural elements has been linked with decreased levels of diastolic blood pressure, depression and anxiety, increased attentional capacity, better recovery from job stress and illness, increased psychological well-being and increased pain tolerance.<sup>1,2</sup> The incorporation of plants in the work environment is linked with improved employee morale, decreased absenteeism and increased worker efficiency and job satisfaction.<sup>1,3</sup> The incorporation of water into built spaces can also relieve stress, promote satisfaction and enhance performance.<sup>4</sup> Access to outdoor green spaces is associated with a range of short- and long-term mental health benefits for individuals across the life span, from children to older adults.<sup>5-7</sup> The many associated benefits include lower levels of anxiety and depression, as well as improved mental recovery from stress and fatigue.<sup>8</sup>

**Solutions :** A dose-response relationship has been found with exposure to indoor nature, with research finding that as workday nature contact increased, perceived job stress, subjective health complaints and sickness absence decreased.<sup>9-11</sup> Researchers also hypothesize that access to outdoor green spaces impacts mental health and well-being through cognitive restoration, and that physiological and emotional changes take place, when individuals are exposed to natural settings.<sup>12</sup> The benefits of nature access have been found across diverse age groups, and supporting access to outdoor green and natural spaces can support the health and well-being of a wide range of individuals within a built community.<sup>13,14</sup>

### PART 1 PROVIDE NATURE ACCESS INDOORS (MAX : 1 PT )

*For All Spaces:*

The project's floor plan is designed such that at least 75% of workstations, conference room seats, classroom seats and seating within common spaces meets at least one of the following requirements:

- a. Have a direct line of sight to indoor plant(s), water feature(s) and/or nature view(s).
- b. Are within 10 m of indoor plant(s), water feature(s) and/or nature view(s).

**Note :**

Water features that may produce water aerosols (e.g., decorative fountains) cannot be installed in hospitals, healthcare settings or senior living facilities.<sup>15</sup>

### PART 2 PROVIDE NATURE ACCESS OUTDOORS (MAX : 1 PT )

*For All Spaces:*

*1: Outdoor nature*

One of the following requirements is met:

- a. Outdoor nature access facilitated by the conditions below:
  1. One or more outdoor spaces that collectively total at least 5% of the project interior area are available to all regular occupants.
  2. At least 70% of the outdoor space must be covered by plants, water or other natural elements. Use the area of each tree or shrub canopy to calculate their contributions to the total area.
- b. Nearby nature access facilitated by the conditions below:
  1. At least one green space or blue space is within a 200 m walk distance from the project boundary and available to all regular occupants during open hours of the space(s).
  2. Total combined green space must be at least 0.5 hectare.<sup>16</sup>

*2: Outdoor nature access*

The following requirement is met:

- a. Occupants are encouraged to access outdoor nature (e.g., presence of signage or maps to outdoor nature, availability of breaks during the workday to go visit outdoor nature).

**Note :**

Interiors projects may count base building amenities towards feature requirements.

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## M10 TOBACCO CESSATION | O (MAX : 3 PT )

**Intent :** Reduce the use of tobacco through interventions that support tobacco cessation and prevent the sale and advertisement of tobacco products.

**Summary :** This WELL feature requires projects that sell retail goods to restrict sale and marketing of tobacco products and supports employee access to tobacco cessation support programs.

**Issue :** Tobacco is responsible for an estimated six million deaths per year globally among direct users, and serves as the cause of death for up to half of its users.<sup>1,2</sup> In addition to those deaths caused by direct use, an estimated 890,000 annual deaths can be attributed to non-user exposure to second-hand smoke.<sup>1</sup> In the workplace, employees that smoke incur greater absences, take more sick days and have higher health care costs than non-smoking employees.<sup>3</sup> Despite tobacco's impact, national comprehensive health services that fully or partially cover services to support tobacco cessation are only available in 24 countries, benefiting just 15% of the world's population.<sup>1,4</sup> Although 70% of U.S. adult smokers are interested in quitting and 40% of smokers attempt to quit each year, only 8% of smoking employees report that their workplace offers smoking cessation assistance.<sup>5-7</sup>

**Solutions :** Employers can play a key role in supporting employee tobacco cessation efforts.<sup>7</sup> Among those who attempt to quit, counseling and medication more than double the chance of quitting success.<sup>1</sup> Worksite-based incentives and competitions, when combined with additional interventions to support individual cessation efforts, can be effective in reducing tobacco use among workers.<sup>8</sup> Another influencing factor on tobacco use is an individual's proximity to outlets where it is sold.<sup>9</sup> Restricting the sale of tobacco on-site is a key strategy for preventing or curbing use of tobacco products, as well as providing support to those trying to quit.<sup>9,10</sup>

### PART 1 PROVIDE TOBACCO CESSATION RESOURCES (MAX : 2 PT )

*For All Spaces:*

*1: Incentive program and cessation resources*

The following requirements are met:

- a. The project or organization implements a tobacco cessation program for all eligible employees that meets the following:
  1. Focuses on increasing or improving motivation or action to quit, or maintaining quit effort.<sup>8,11</sup>
  2. Includes incentives or rewards (e.g., direct financial payments, lottery for prizes) provided for participation in quit effort or success in abstaining from tobacco use.<sup>8,11</sup>
- b. Tobacco cessation resources are available to all eligible employees at no cost or are subsidized and meet the following:
  1. Resources referring tobacco users to tobacco cessation telephone quit lines or online quitting resources.<sup>11</sup>
  2. Tobacco cessation counseling. Programs may be provided in-person or virtually; in group or individual settings; through vendors, on-site staff, health insurance plans or programs, community groups or other qualified professionals (e.g., tobacco cessation specialist).<sup>11</sup>
  3. Prescription tobacco cessation medications and nicotine replacement products (e.g., inhalers, nasal sprays, bupropion, varenicline).<sup>11</sup>
  4. Over-the-counter nicotine replacement products (e.g., nicotine gum, patches, lozenges).<sup>11</sup>

### PART 2 LIMIT TOBACCO AVAILABILITY (MAX : 1 PT )

*For Retail Spaces:*

The following requirements are met for projects where retail products are sold on a daily basis:

- a. Sale of tobacco products (including e-cigarettes) is prohibited.<sup>11</sup>
- b. Tobacco products (including e-cigarettes) are not marketed or promoted.<sup>12</sup>

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## M11 SUBSTANCE USE SERVICES | O (MAX : 2 PT )

**Intent :** Increase availability and access to substance abuse and addiction support services, resources and care and prevent the development of substance abuse and addiction among occupants.

**Summary :** This WELL feature requires projects to outline policies regarding drug and alcohol use in the workplace, provide education on substance use and addiction and support access to substance use services.

**Issue :** Alcohol and drug use contribute significantly to the global burden of premature death and disability.<sup>1</sup> Harmful use of alcohol is a leading global risk factor, accounting for 3.3 million deaths per year (or 5.9% of all deaths) and 5.1% of the global burden of disease in 2014.<sup>2</sup> Within the workplace, alcohol use is a significant risk factor for absenteeism, presenteeism, accidents and employee turnover.<sup>3,4</sup> It is also estimated that approximately half of the overall social costs of alcohol are due to lost productivity.<sup>5</sup> In addition to alcohol, illicit drug use remains a serious global public health concern: in 2013, 246 million people between 15 and 64 years of age used illicit drugs, of which one in ten suffered from a drug use disorder or dependency.<sup>5</sup>

**Solutions :** Prevention programs that address substance use through education and workplace policy, as well as available and affordable screening and treatment offerings, have been shown to be effective methods of preventing unhealthy substance use habits and supporting those struggling with substance abuse and addiction.<sup>3,4,6</sup> The return on investment in offering substance use treatment and prevention services is known. For example, each \$1 invested in screening and brief counseling interventions saves approximately \$4 in health care costs.<sup>7</sup> Offering prevention, education and support services provides an opportunity to reduce the costs companies face as a result of undiagnosed and untreated substance abuse.<sup>7</sup>

### PART 1 OFFER SUBSTANCE USE EDUCATION (MAX : 1 PT )

*For All Spaces:*

*1: Substance use policy and education*

The following requirements are met:

- a. A policy is in place regarding the use of alcohol, legal and recreational drugs on-site and is clearly communicated to all regular occupants.<sup>7</sup>
- b. Trainings (in the form of education seminars, workshops or classes) are offered at least once per year to regular occupants and addresses the following topics:
  1. Management of personal substance use, covering, at minimum, safe substance use habits, signs of dependency or addiction and short- and long-term health risks associated with substance misuse or addiction.<sup>8,9 2.</sup>
  2. Prescription opioid education, covering, at minimum, questions to ask at point of prescribing, safe use (e.g., storage, disposal, driving while using) and risks and signs of dependency or addiction.<sup>9 3</sup>
  3. How to appropriately respond to a peer struggling with substance use, covering, at minimum, how to support a peer's recovery efforts and what to do in the case of relapse or a substance use emergency (e.g., withdrawal, overdose).<sup>8</sup>
- c. Training is provided in-person or virtually; in group or individual settings; through vendors, on-site staff, health insurance plans or programs, community groups or other qualified practitioners or programs (e.g., Mental Health First Aid).<sup>10</sup>

### PART 2 PROVIDE SUBSTANCE USE AND ADDICTION SERVICES (MAX : 1 PT )

*For All Spaces:*

The following requirements are met for all eligible employees:

- a. Substance use and addiction services are available at no cost or subsidized and include at a minimum:
  1. Clinical screening and referral to licensed mental health professionals and support resources.<sup>7</sup>
  2. Counseling services, including telemental health services (e.g., online behavioral therapy).<sup>7</sup>
  3. Outpatient treatment (e.g., day programs).<sup>7</sup>
  4. Inpatient treatment (e.g., residential programs, hospitalization).<sup>7</sup>
  5. Medication-assisted treatment (e.g., methadone treatment).<sup>7</sup>
- b. Organizational commitment to mental health parity in health service coverage.<sup>8</sup>
- c. Information on benefits coverage and how to access substance use and addiction services and community resources (e.g., peer support groups, online support groups) is easily and confidentially available (e.g., via a health portal or employee website).<sup>7,8</sup>
- d. Confidential benefits consultation is available with clearly identified and qualified support staff (e.g., benefits counselor, human resources representative).

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